

Crossroad between current knowledge and new perspective of diagnostic and therapy of late-onset schizophrenia and very late-onset schizophrenia-like psychosis: An update.

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Schizophrenia is a chronic, highly individualized disease with many symptoms that can occur with varying severity in different patients. Schizophrenia affects 1% of the population, but occurs in almost 20% of patients after 40 years of age. It should be noted that the next peak in the incidence of schizophrenia occurs at the age of 60 years, affects mostly females, and is closely associated with a high risk of developing memory disorders. Therefore, postadolescent schizophrenia includes two distinct groups of patients: those whose symptoms onset at the age of 45 or 60. The purposes of this literature review were as follows: (1) synthetically characterize the clinical manifestations of schizophrenia; (2) discuss difficulties in the diagnosis of schizophrenia, especially in patients over 40 years of age; (3) discuss the clinical utility of different classes of marker in diagnostic and differentiating schizophrenia from neurodegenerative diseases in elderly people; (4) discuss therapeutic options for schizophrenia, pharmacotherapy, and psychotherapy, emphasizing the role of caregivers of people with psychosis in therapy, in preadolescence and postadolescence schizophrenia. We have tried to primarily discuss the findings of original articles from the last 10 years with an indication of their clinical implications with the issues discussed in the various subsections. Moreover, despite many years of research, no specific, precise algorithm has been developed that can be used in clinical practice during the diagnosis of schizophrenia. For this reason, the diagnosis of schizophrenia is primarily based on an interview with the patient and his family, as well as on the experience of a psychiatrist. It also seems that schizophrenia treatment should be carried out holistically, including pharmacotherapy, psychotherapy, and the support of caregivers of patients who have this psychosis, which increases the achievement of therapeutic success. Finally, we must be aware of the difficulties in diagnosing schizophrenia in the elderly and the need to modify pharmacological treatment. Currently, no guidelines have been developed for the differentiation of negative symptoms in elderly patients with schizophrenia from amotivation/avolition/apathy symptoms in elderly patients with neurodegenerative disorders.

Antipsychotic drug treatment for elderly people with late-onset schizophrenia.

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Schizophrenia is usually considered an illness of young adulthood. However, onset after the age of 40 years is reported in 23% of patients hospitalised with schizophrenia. At least 0.1% of the world's elderly population have a diagnosis of late-onset schizophrenia which seems to differ from earlier onset schizophrenia on a variety of counts including response to antipsychotic drugs. To assess the effects of antipsychotic drugs for elderly people with late-onset schizophrenia. We searched the Cochrane Schizophrenia Group Trials Register (January 2010) which is based on regular searches of CINAHL, EMBASE, MEDLINE and PsycINFO. We inspected references of all identified studies for further trials. We contacted relevant authors of trials for additional information. All relevant randomised controlled trials that compared antipsychotic drugs with other treatments for elderly people (at least 80% older than 65 years) with a recent (within five years) diagnosis of schizophrenia or schizophrenia like illnesses. For the 2010 search, two new review authors (AE, AG) inspected all citations to ensure reliable selection. We assessed methodological quality of trials using the criteria recommended in the Cochrane Handbook for Systematic Reviews of Interventions. AE and AG also independently extracted data. For homogenous dichotomous data, we planned to calculate the relative risk (RR) and 95% confidence interval (CI). There were no included studies in the original version of this review (2002 search). The 2010 search for the

current update produced 211 references, among which we identified 88 studies. Only one study met the inclusion criteria and was of acceptable quality. This was an eight-week randomised trial of risperidone and olanzapine in 44 inpatients with late-onset schizophrenia. All participants completed the eight-week trial, indicating that both drugs were well tolerated. Unfortunately, this study provided little usable data. We excluded a total of 81 studies, 77 studies because they either studied interventions other than antipsychotic medication or because they involved elderly people with chronic - not late-onset - schizophrenia. We excluded a further four trials of antipsychotics in late-onset schizophrenia because of flawed design. Five studies are still awaiting classification, and one is on-going. There is no trial-based evidence upon which to base guidelines for the treatment of late-onset schizophrenia. There is a need for good quality-controlled clinical trials into the effects of antipsychotics for this group. Such trials are possible. Until they are undertaken, people with late-onset schizophrenia will be treated by doctors using clinical judgement and habit to guide prescribing.

Historical development of psychiatric classification and mental illness.

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Psychiatric classification has been developing for over 100 years and there are now over 400 diagnoses available. Dementia and adjustment disorder were the only diagnosis available for older adults until a number of papers published between 1944 and 1955 demonstrated the relevance of functional mental illness for older adults. It was not until 1987 that the Diagnostic and Statistical Manual published by the American Psychiatric Association included an option to diagnose schizophrenia in the over 45s. This paper describes the historical development of psychiatric classification and discusses how this has potentially influenced ageist attitudes to older people, with particular reference to schizophrenia and dementia. Nurses are encouraged to reflect on their own attitudes in light of this discussion as to how medical frameworks for understanding mental illness may have influenced personal beliefs, particularly ageist beliefs about older adults.

[Schizophrenia and delusions in middle aged and elderly patients. Epidemiology and etiological hypothesis].

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Knowledge of the similarities and differences between early- and late-onset schizophrenia and between late-onset schizophrenia and paranoid disorder of old age and very old age is fragmentary. We compared diagnosis, subtypes, syndromes and symptoms between first-episode schizophrenia (ICD-9: 295) and paranoid disorder (ICD-9: 297, 298.3/4.) over the life cycle in a population-based (N = 232) and a clinical first-admission sample (N = 1109). Apart from different age patterns of the sexes only two symptom groups were significantly different between early- and late-onset illness: paranoid and systematic delusions showed a linear increase, symptoms of disorganisation a linear decrease over the life cycle. Clearly different between early- and late-onset illness were the neurobiological and psychological risk factors, suggesting that both neurodevelopmental and neurodegenerative disorder causes psychopathology typical of schizophrenia. Late- (40 to 60) and very-late-onset (over 60) cases of both groups of illness showed the same symptom profiles, merely the number of symptoms being higher in the group diagnosed with schizophrenia. Age was the only factor significantly contributing to a

clinico-diagnostic differentiation of schizophrenia from paranoid disorder beyond age 40.

[Clinical psychopathological research on late-onset schizophrenia--mainly patients with schizophrenia from a hospital psychiatric ward].

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In the field of clinical psychiatry, cases of late-onset schizophrenia are often observed in the population of 40 years or older. Female patients seem to significantly predominate those diagnosed with late-onset schizophrenia. Generally, paranoid delusions of reference with family members, neighbors, and friends are observed as clinical features of such late-onset schizophrenia conditions. Medical treatment for such a condition is often effective and considered to improve the prognosis. The authors conducted clinical research at Jichi Medical University Hospital psychiatric ward involving 38 late-onset schizophrenia patients (7 males; 31 females) diagnosed over the age of 40 using DSM-IV-TR diagnostic criteria. Subjects were selected from 316 schizophrenia patients (164 males; 152 females) admitted to the hospital for schizophrenia treatment at some time during the 13 years from April 1, 1993 to March 31, 2006. Also, another 14 late-onset schizophrenia patients diagnosed over the age of 40 (1 male; 13 females), with additional investigation, were selected from 130 cases (50 males; 80 females) treated in related facilities at some time during the 2 years from April 1, 2004 to March 31, 2006. The investigation revealed the following results: (1) Cases showing an onset after the age of 40 comprised 12% of the total population. Female cases comprised 20.4%, being significantly higher than that of male cases (4.3%). Within the psychiatric ward, cases showing an onset after 40 made up 10.8% of the total population. Female cases comprised 16.3%, being significantly higher than that of male cases (2.0%). (2) The paranoid type comprised 55.3% of the total population of late-onset cases, being significantly higher than in early-onset cases younger than 40 years old. A total of 55.3% of late-onset cases also showed depressive symptoms, being significantly higher than in early-onset cases. (3) For late-onset, 55.3% of patients showed an introverted premorbid character, while 15.8% of patients exhibited an extroverted premorbid character. Regarding late-onset cases, the number of introverted premorbid character cases significantly lower than in early-onset cases diagnosed at 40 years or younger; however, no significant difference was observed for the extroverted premorbid character. (4) A total of 65.8% of the population of late-onset cases were diagnosed as having psychosocial stresses as their cause. In addition, 36 % of subjects with psychosocial stress recognition had experienced a sense of loss. Of these, 66.6% of the loss experience involved separation from their family members. (5) Cases showing a successful recovery from schizophrenia comprised 55.3 % of the total population. On the other hand, cases with an unsuccessful outcome were observed in 34.2% of the total population, accounting for a relatively large portion. A total of 30.8% of subjects with an unsuccessful outcome were unmarried at the time of onset, and made up 57.1% of the late-onset population. According to the results of this investigation, late-onset schizophrenia represents about 10% of the total cases, and female cases are significantly more common than male cases. For late-onset patients, subjects' self-functions were more developed compared to those of early-onset patients, so they did not seem to clearly show early-onset symptoms during a younger period. The late-onset type has a tendency to show a better rate of successful recovery; however, there are situations where the condition reverts toward the "disorganized type", moving closer toward Kraepelin's early-onset disorganized schizophrenia.

Late-onset psychosis and very-late-onset-schizophrenia-like-psychosis: an updated

systematic review.

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Psychotic disorders have long been known to be a condition that peaks during adolescence and early adulthood. A considerable proportion of patients have their first onset at or after the age of 40, but little is known about this population. The current systematic review examined the clinical presentation of late-onset psychosis (LOP) and very-late-onset-schizophrenia-like psychosis (VLOSLP) with focus on their psychopathological, neuropsychological, neurobiological, psychosocial and psychological correlates. A systematic search of studies published from 2000 to 2019 from Cochrane Library, Pubmed, Medline, Embase, PsycINFO, and Scopus yielded 27 original studies that were included in this review. Results revealed there is a dearth of empirical research on the conditions in the current literature and inconsistencies in the findings reported may be associated with the lack of uniformity in the definitions for LOP and VLOSLP. Future research on the topic shall (i) specify the onset age criteria for LOP and VLOSLP; (ii) study the conditions independently; (iii) involve a larger sample size, and iv) account for potential confounding variables. A comprehensive evaluation of the risks and benefits of pharmacological treatment may also be needed.

Very Late-onset Schizophrenia Like Psychosis: Case Series and Future Directions.

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The quest to unravel the mysteries of schizophrenia has led to immense research in this area over the years. Previously schizophrenia was considered to strictly be an early onset disorder. However, the heralding of baby boomers, with a subsequent surge in the elderly population, has led to growing curiosity in the geriatric age group with schizophrenia. The nosology of late-onset and very late-onset schizophrenia like psychosis (VLOSLP) has remained contentious. The international late-onset schizophrenia (LOS) group has proposed that schizophrenia with an onset between ages 40 and 60 be termed LOS and above 60 years termed VLOSLP. We present two case reports of VLOSLP and its relevance to the present day context.

An electroencephalographic investigation of late-onset schizophrenia.

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We performed awake and resting electroencephalograms (EEGs) on 22 subjects with DSM-III-R schizophrenic disorder of late onset (at or after age 50 years), and compared them with the EEGs of 33 healthy community controls matched for age and gender. The EEGs were rated qualitatively and a 2-minute, artifact-free tracing from each subject was quantified manually by an experienced neurophysiologist unaware of the identity of the subject group. The only significant difference was the presence of more generalized slowing in the EEGs of schizophrenia patients, which was at least partially accounted for by the effect of neuroleptic drugs. The schizophrenic subjects did not have a greater prevalence of epileptiform disturbances or abnormal asymmetry of the EEG compared to the control group. Our study does not suggest the presence of underlying dementia in schizophrenia of late onset.

Late-onset-psychosis: cognition.

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The objectives of the study were to characterize and compare the cognitive profile and natural evolution of patients presenting late-onset psychotic symptoms (LOPS: onset $\geq$ 50 years old) to those of elderly patients ($\geq$ 50 years old) with life-long/early-onset schizophrenia (EOS: onset $<$ 40 years old). Neuropsychological profiles of 15 LOPS patients were compared to those of 17 elderly EOS patients and to those of two control groups ($n = 11/\text{group}$). The evolution of the two patient groups was compared using an independent diagnostic consensual procedure involving a geriatric psychiatry physician/clinician and a neuropsychologist blinded to the initial psychiatric diagnosis. EOS presented significant memory and executive impairments when compared to controls but there was no significant difference between LOPS and their controls when age and education were taken into account. However, a detailed inspection of normative data suggests more executive impairments in LOPS than in EOS. The clinical judgment of experts was in favour of significant cognitive deficits with or without dementia in most LOPS (82.3%-94.1%) and EOS (80.0%-93.3%) patients. Regarding evolution, mild cognitive impairment (MCI) and vascular cognitive impairment (VCI) were the most common clinical diagnoses made by geriatric psychiatry physicians/clinicians for the LOPS (40%). In addition, 20% of LOPS versus 5.9% of EOS patients met the diagnostic criteria for dementia by consensus of the experts. Cerebral abnormalities were confirmed (CT scan; SPECT) in 73.3% of LOPS patients. The present results suggest cognitive deficits (mostly of executive functions) and vascular and neurodegenerative vulnerability in LOPS. Further studies with larger samples are needed to confirm the present findings.

Focus on geriatric psychiatry: schizophrenia in later life: clinical symptoms and social well-being.

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The number of persons aged 55 and older with a diagnosis of schizophrenia is projected to double over the next 20 years. A tripartite classification system of early-onset schizophrenia, late-onset schizophrenia, and very-late-onset schizophrenia-like psychosis has been proposed. This column reviews recent findings on the outcome and associated features of clinical symptom and social well-being categories for older adults with early-onset schizophrenia.